Adjuvant ribociclib + endocrine therapy

The usual adjuvant starting dose is ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to three years. The aromatase inhibitor continues every day on its own plan; ovarian suppression is added when indicated.

· selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart rhythm / QT

Ribociclib

Baseline electrocardiogram (ECG) and an ECG around Cycle 1 Day 14. Fainting or palpitations need urgent advice.

2 Liver injury

Ribociclib

Keep liver blood tests. Yellow skin / eyes, dark urine or marked upper-abdominal discomfort: call promptly.

3 Low neutrophils / infection

Ribociclib

Blood counts guide whether the next cycle can start. Fever, shivers or sudden illness: contact oncology immediately.

4 Lung inflammation

Ribociclib · uncommon but important

New cough or breathlessness needs prompt assessment, even between visits.

5 Severe rash or blistering

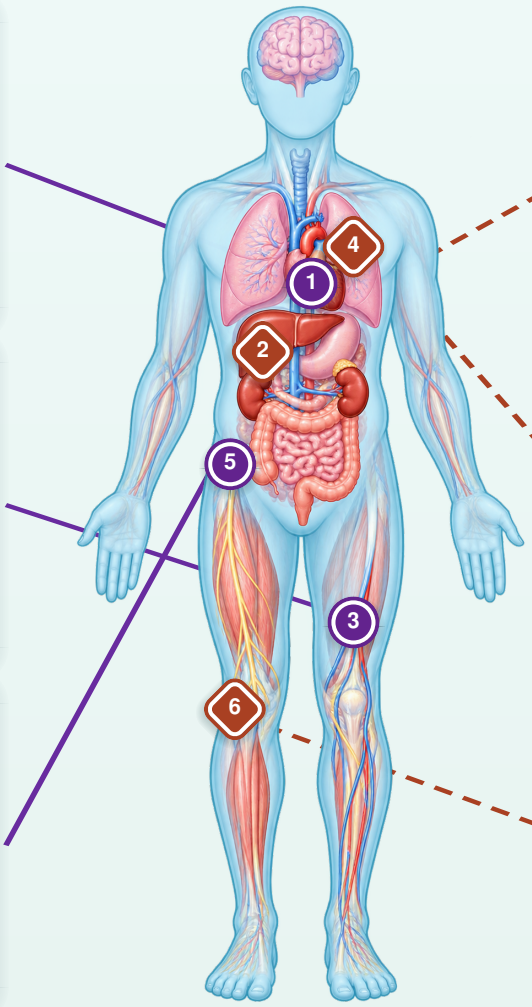
Ribociclib

A widespread painful rash, blisters or peeling skin needs urgent assessment.

6 Joints and bone health

Aromatase inhibitor / ovarian suppression

Discuss joint symptoms and keep the agreed bone-risk and dual-energy X-ray absorptiometry (bone density) (DXA) plan.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

Know the next step.

Taking treatment

The usual adjuvant starting dose is ribociclib 400 mg once daily on Days 1–21, followed by Days 22–28 off, for up to three years. The aromatase inhibitor continues every day on its own plan; ovarian suppression is added when indicated.

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription.

Checks and follow-up

full blood count (FBC), renal and liver profile; potassium, calcium, magnesium and phosphorus. ECG before initiation; QTcF must be below 450 ms. Correct electrolytes and review QT/CYP3A interactions. Confirm stage, recurrence-risk criteria, prior treatment, aromatase inhibitor (AI) and ovarian-suppression plan; reconcile the NCCP node-negative eligibility/exclusion conflict before selection. FBC, renal and liver profile every two weeks for cycles 1–2, then before cycles 3–6, then as clinically indicated. Electrolytes before each of the first six cycles, then as clinically indicated. Repeat ECG around cycle 1 Day 14, then as clinically indicated. Increase liver monitoring for grade 2 or worse abnormalities; review fever, jaundice, palpitations/fainting, breathlessness and severe skin symptoms. Keep early-breast surveillance and bone/endocrine follow-up distinct from treatment-toxicity checks and from metastatic response imaging.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Nausea support

NCCP classifies ribociclib as oral moderate–high emetic risk. Agree preventive or rescue treatment under local policy after QT and CYP3A interaction review; no default QT-prolonging antiemetic is selected by this information card.

Heart rhythm and electrolytes

Attend the baseline and cycle 1 Day-14 ECG and scheduled electrolyte checks. Fainting, palpitations or severe dizziness need urgent assessment.

Blood and liver monitoring

Keep two-weekly blood tests during cycles 1–2 and pre-cycle tests during cycles 3–6. Report fever, jaundice, dark urine or new breathing/skin symptoms promptly.

AI and ovarian-suppression support

The aromatase inhibitor continues during the ribociclib rest week and generally after ribociclib completion. Discuss menopause, joints, sexual health, bone risk and any injection-site symptoms from ovarian suppression.

Completion and follow-up

Ribociclib has a planned maximum three-year course. Confirm the endocrine duration and early-breast surveillance plan separately; new symptoms should be assessed without waiting for a routine appointment.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital's urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00892/

Public information only. No personal information is encoded.